

HOPE Program
Preparatory Treatment Manual

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ABOUT THE HOPE PREPARATORY TREATMENT PROGRAM

The HOPE Preparatory Sexual Offender Treatment Program (Prep Program) is designed for individuals who have been convicted of a sexual offense or have engaged in inappropriate or compulsive sexual behaviors and who have realized that they need help in addressing their behavior(s).

The Prep Program is a 24 session (12-week) open/rolling program for adult males who have been convicted of a sexual offense. It is a cognitive-behavioral treatment program that is primarily based on the Risk Need Responsivity (RNR) model (Andrews & Bonta, 2003). As this is a preparatory treatment program, considerable emphasis is placed on motivation and engagement as the primary means with which to engage offenders and prepare them for success in the core treatment program as well as discharge.

In the Prep Program, it is essential that participants be introduced to the goals of treatment, what they should expect from treatment, the theoretical orientation of treatment, and that they understand and build on motivational aspects of the assessments conducted with them. In addition, as research indicates, at the outset of treatment, group facilitators must build a strong therapeutic/working alliance with the group members. A number of studies have demonstrated that the therapeutic alliance predicts outcome and is a fundamental tool for achieving client change (Martin et al., 2000; Marshall et al., 2011; Norcross, 2002), and that this is essential to ultimate outcome.

In general, the group facilitator should adopt a reinforcing and encouraging approach, while also being firm and setting limits (see appendix for effective group and therapist characteristics). This style should be evident throughout the treatment program but is particularly important at the beginning stages of therapy when clients may have low self-efficacy and some resistance to change. Motivational enhancement strategies are essential at this stage of treatment. It is also important that participants invest in the treatment and group process in order to encourage openness and disclosure and to allow participants to respectfully challenge each other. Lastly, it is essential that participants understand how this treatment program can benefit them and help them to achieve their goals in life.

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It is essential that the facilitator convey the Good Lives approach as well as other relevant modalities utilized in this program. An essential message to convey is what participants will gain in their lives by participating in treatment; that is, how will they benefit from treatment, beyond managing their risk to re-offend. In brief, this program is designed to help participants to both not to commit a new sexual offence and to help them discover how to have a better life in areas outside sexual offending.

The Prep Program is conceptualized as having two phases. Phase 1 is directed at providing individuals with the knowledge of HOPE Program policy and procedures as well as relevant treatment concepts that will prepare them for success in the subsequent program and following discharge. Phase 2 is primarily directed at motivating and engaging clients, establishing appropriate group climate, and beginning the development of an effective therapeutic relationship. Initial issues discussed should be non-threatening although a gradual degree of supportive challenging is recommended.

The initial issues to be addressed include a description of the rules of the group, an outline of the program (both the Prep and Core treatment curriculum), and an indication of the general findings of the effectiveness of sexual offender treatment. The subsequent targets of the Prep program include the Good Lives Model, a cost-benefit analysis, an autobiography, and recognition of immediate offense triggers. Relevant topics are included in this manual for discussion amongst the group.

Typical Session Outline



I. Check In. At the beginning of each session, there is a 10-minute check-in to discuss issues that have arisen between session, to assist group members with homework or assigned tasks, or to discuss concerns related to their personal circumstances. It is important to not let this discussion go on for more than 10-minutes except in unusual circumstances. Important issues will be discussed in each session.

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It is generally not recommended that more than two people present more elaborate assignments, such as the autobiography and offense disclosure exercise in one session to allow for adequate discussion time.

II. Feedback. All participants are expected to initially provide positive feedback to the group member who is presenting his exercise prior to asking questions about the details. Following the positive feedback, clients are encouraged to ask questions about the assignment. It will be up to the facilitator to ensure the content remains positive and, when relevant, supportively challenging.

III. Homework: Homework is to be completed between sessions and is an essential component of treatment completion. The importance of homework and the overall completion criteria must be discussed with the group in the initial session. To successfully complete the prep program and transition to the core program, an individual must actively participate in the program, complete all assigned homework, and present their autobiography and offence disclosure exercises. Of the 24 sessions, they are only permitted to miss two sessions and these must be for valid reasons and be consistent with HOPE policy (e.g., 24-hour advanced notice from the agent; 48-hours from the client).

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MODULE 1: Introduction to Treatment

Key Learning Points/Module Goals

- To provide individuals an overall orientation to the program
- Participant and clinician introduction
- Establish rapport

**** As this is an open/rolling group, whenever a new client starts the Prep Program, the facilitator should spend a sufficient proportion of the session introducing the new group member to the other clients, the curriculum, and the relevant policies and procedures. After introductions, an overview of the prep and core programs should be presented, as well as the group rules and how to give and receive feedback. It is strongly recommended that the facilitator have the group members explain these various concepts to the new member and then missing/relevant information can be supplemented by the facilitator.**

Phase 1 represents the introductory steps in treatment. Although it is primarily psychoeducational, it is important to note that the initial phase of any treatment program should be focused on winning the confidence and trust of the client and establishing a strong therapeutic alliance. The content of these initial sessions needs to be presented every time a new group member stats the program.

During the initial session, the facilitator welcomes everyone to the HOPE Program and introduces the overall HOPE approach toward treatment. It is essential to note that while this program targets risk for sexual offending, it is much broader in scope and is designed to help participants to not commit a sexual offense and help them have a better, more fulfilling life in areas outside of sexual offending. Evidence of the effectiveness of treatment should be presented. There have been several large-scale meta-analyses of treatment outcome with sexual offenders with virtually identical results. Over a 5-year follow-up period, official recidivism data revealed re-offense rates of approximately 9% for the treated group versus 18% for the untreated group. Some specific programs have even shown better rates. Other programs that emphasize motivational and process, such as the current program have shown even better results, such as 3.2% for treated sexual

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offenders versus 17% for untreated sexual offenders (see Marshall et al., 2011). The goal of this presentation is to establish hope and self-efficacy early on in the treatment process.

Clients are also told that in order to determine the aspects of knowledge, skills, attitudes, and self-belief they need to acquire to build toward the attainment of a fulfilling life, we must examine in detail what was happening in their life prior to their offense and what their life history has been to this point. This will be discussed in this program but will be followed up in greater detail in the subsequent modules/blocks.

Next, the facilitator reviews key concepts in the paperwork they signed. Notably, the limits of confidentiality and group rules. The discussion of group rules should be initially an interactive process, such that most of the group rules should be generated by group members and key concepts written on the whiteboard. Socratic style questioning should be used throughout this exercise. After discussion, the official HOPE Program group rules should be presented and any differences or omissions should be highlighted. Other relevant policies and procedures should be discussed. These include excused/unexcused absences, differences between HOPE rules and parole rules, HOPE contact information, treatment dosage, etc.

HOPE Group Rules

1. Be on time. If you are more than 10 minutes late, you will not be allowed into group and will be marked unexcused.
2. Repeated unexcused absences will result in being evaluated for termination from the program.
3. Be quiet around the HOPE Program. No loud music, keep your voices quiet. Do not loiter. You cannot smoke within 30 feet of the building.
4. Respect group members' confidentiality. What is said in group stays in group.
5. Group members are not to have contact with each other outside the group.
6. Any issues that arise should be discussed in a group or individual session, not outside.
7. No food or candy allowed in group. Beverages with covers are allowed. Throw away your trash.

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8. If you bring a cell phone into group, it must be put in the bucket and/or turned off prior to entering the group. If you are caught with a cell phone in group, you will be removed from group.
9. Speak to others with respect, no physical contact permitted.
10. Group members are not allowed to come to group under the influence of drugs or alcohol.
11. No weapons allowed on Hope Program Campus.
12. Once comfortable, Group members must participate in group and complete their homework.
13. Group facilitators will disclose the reason for group members' absences if group members are terminated or discontinue treatment.
14. Group members are not permitted to leave group once the group session begins. This includes bathroom breaks or getting water. Medical exceptions are permitted for valid medical reasons with a doctor's note.

Introductions of Participants and Group Leader(s)

Everyone in the group should introduce themselves (the group leader should do this first) and mention something that they are good at or that they like to do (e.g., enjoy watching movies). Group members should be encouraged to ask questions about each other's hobbies and interests (e.g., what types of movies do you like to watch?).

The facilitator should focus the introductions on positive content (e.g., what are you good at). This will help establish feelings of comfort in the group setting and will begin the process toward a strong therapeutic alliance, as well as orient participants to the Good Lives Model and this approach to treatment. Encouraging the group members to ask appropriate questions will also begin to establish group cohesion and set the stage for positive interactions in subsequent sessions.

Group facilitators should ask about group members' perceptions of sexual offender treatment and their own feelings about being in the group. Facilitators should utilize Socratic questioning to elicit group members' expectations and to explore how each person feels about being in the group. Common feelings may include anxiety, anger, fear, excitement, etc. Facilitators should validate group members' feelings and reinforce their

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participation in treatment and their motivation to make some changes toward a happier and healthier life.

Facilitators should be alert to any inappropriate or negative statements. For example, group members who use labels such as “sex offender”, “pedophile”, “pervert” etc., should be gently but firmly challenged. Group members are encouraged to view themselves as much more than their behavior and the use of the term “someone who committed a sexual offense” is discussed. Additionally, any antisocial attitudes and cognitive distortions should be challenged only to a minimal extent at this stage. Facilitators should challenge such statements but be mindful that it is early in the process and that the focus should be on establishing trust and rapport.

Group Cohesion Exercise



The “*I am*” game. Facilitators write a number of positive affirmation words/statements on the whiteboard (e.g., funny, friendly, nice, a good friend, etc.). Each group member should say aloud, “*I am* _____” (insert one of the words). Each participant explains the reasons they chose this word. The facilitator should model appropriate questions to probe about examples where they demonstrated this strength, and reinforce positive and appropriate responses, while gently challenging and redirecting negative or inappropriate responses. Group members should be encouraged to do this as well.

This group activity is intended to be enjoyable and to increase cohesion and trust among the group members. This exercise is also intended to increase self-esteem which, although not a criminogenic need, is important for group cohesion and engagement in the therapeutic process.

Self-Esteem Homework Exercise



1.Write out 5 positive things about yourself. These positive features do not need to describe exemplary features, but rather everyday acceptable features such as “I am a reasonably generous person”, “I am a reasonable interesting person”, “I am a good worker”, “I am a loyal friend”.

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NOTE: some clients may find this exercise extremely difficult. Although it is good to be sensitive and acknowledge their difficulty, do not allow the group member to not complete the exercise, even if it takes several sessions of between-session work by the client. For some clients you may need to work on one domain at a time and slowly build their list.

- *Work*
- *Social & Friends*
- *Leisure & Sport*
- *Intimate Relationships*
- *Knowledge*
- *Personal Characteristics*
- *Appearance*

2. Help the client identify social activities he can engage in. Have him increase the frequency with which he enters and participates in these social activities. Help the client identify, and become sensitive to, everyday pleasurable or satisfying experiences (e.g., having a shower, engaging in exercise, completing a hobby). Have the client increase the range and frequency of pleasurable and appropriate activities over time.

Closing the Session

Review the major points of the session (program overview, group rules/expectations, confidentiality). Provide a brief discussion about what they can expect in the next session. Emphasize that many people have attended these types of groups and that if they put in the effort in group that they can expect positive outcomes as a result of treatment. Again, link this back to achieving goals and living a positive and healthy life, and what participants will gain (i.e., a better life) from their participation in the group. The purpose of this discussion is also to increase hope for the future.

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MODULE 2: Types of Groups and Feedback

Key Learning Points/Module Goals

- Assist individuals in being able to understand the importance of the group process and group dynamics
- Familiarize individuals with the various types of group dynamics that can form during the group process
- To orient clients on how to give and receive feedback

Rationale

The purpose of this module is for group members to gain an understanding of the group process and how different types of group dynamics may form amongst members, and the pros and cons of each group dynamic. The Barn Raising group is the most beneficial for individual and group growth as each member is supportive but constructively challenging to other members of the group. In this group dynamic, members are held accountable for their cognitive distortions and high-risk behaviors. Thus, it is important for individuals to understand what behaviors support growth among members.

It is also fundamental to a good group on how best to provide and receive feedback. This will serve as the foundation of the two core assignments presented later in this manual and is the primary mechanism of change.

Types of Groups

1. Free-For-All Group

In the Free-For-All Group, everyone is out for themselves. They feel scared to talk or to be vulnerable, and they do not trust one another or the facilitators. They do not want to participate or give the facilitators what they want, so they remain quiet or give meaningless answers. Everyone is competing to be the best, the coolest, the least involved. Anyone who participates is mocked or shunned.

Pros: Ego boosting when a person feels like a “winner,” people get to protect themselves from examination or criticism, safety from vulnerability.

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Cons: Hostility or lack of participation can be obstacles to getting help from the group. The group feels unsafe, there is no group cohesion therefore no one works together or feels good about being in group.

2. Beauty Contest Group

In the Beauty Contest Group, everyone understands what the facilitators want and try to give it to them. People compete to be the "favorite" of the facilitators. People try to give "correct" answers to questions and show off their knowledge. People who do not give correct answers are mocked. People who do not participate are shunned.

Pros: Good flow of conversation; people want to talk instead of remaining silent; people care what others in the group think about them and their answers; people understand what the point of the group is.

Cons: Answers are superficial and given to get praise rather than to express honesty; people avoid negative experiences and try to focus only on positive statements; problems are not examined in depth or solved effectively.

3. Club Group

In the Club Group, everyone wants to give positive feedback and get positive feedback in return. People praise one another and point out each other's strengths and successes. Each person gets a turn in the spotlight and gets praised by the group. People who bring up negatives or who ruin the good mood are dismissed and the group quickly shifts the focus back to "positive" feelings. People who express anger or dislike towards peers are shunned. People who don't participate are defended by peers.

Pros: Acceptance from the entire group towards each member; feeling of being a part of group; offering of support to everyone; plenty of socialization and friendliness; support network provided.

Cons: People are nonchalant about actual problems; each member gives praise selfishly in order to receive it back; people are afraid to go deeper into issues; group culture prevents bringing up or examining problems; peer pressure may keep people from speaking up about negative issues; people do not challenge one another or hold one another accountable.

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4. Barn Raising Group

In the Barn Raising Group, everyone works together towards a common goal. Each person takes a part of the task of helping each other person, and no one carries the entire group alone. Each person understands what is being asked of them and takes a turn being examined by the others. Each person is held accountable by the group and their distortions or blind spots are pointed out. When people feel unhappy or badly treated, they bring it up and everyone gets to understand why they feel that way and what can be done about it. Solutions to problems are discussed. The facilitators are seen as allies rather than enemies. People who don't participate are questioned and held accountable.

Pros: Genuine support is offered to each member; alternatives to problematic behaviors are explored; help from everyone increases chances of accomplishing goals; people are encouraged to be genuine and honest.

Cons: Each group member must take more risks, everyone must do more work, each person must allow themselves to be vulnerable. People must face up to negative facts rather than ignoring or sugar-coating them; honesty creates opportunity for conflict which must be resolved; each person bears responsibility for their own well-being as well as participating in the group.



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Experiential Exercise



Facilitators ask the clients to describe what each group looks like and what their (clients) individual role would look like in each of the groups. Have the group discuss what it would take to become a Barn Raising Group. Discuss what obstacles exists to achieving this group and how that would impact each individual's treatment.

- Would it delay their treatment?
- Is participation in group evaluated? Expected?
- Fluctuation between groups is expected but the ultimate goal is to achieve the culture of the Bare Raising Group, how can you personally work towards this?

As a subsequent discussion, the facilitator generates a discussion on how to give and receive feedback, as this is a primary mechanism of intervention in the Prep Program (i.e., core assignments #1 and 2). As with most didactic exercises, this discussion should be as interactive as possible with the group members generating an initial list of appropriate ways to give and receive feedback.

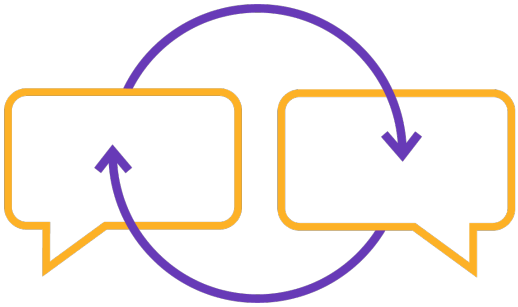
Giving Feedback

1. Think about what it is you want to say before you say it
2. Direct your feedback to the person (maintain eye contact, address the person)
3. Observe your tone of voice and body language (angry/passive)
4. Do challenge appropriately by identifying issues rather than attacking and blaming
5. Do not attack or blame. It may feel satisfying but is not helpful
6. Stay on track – try to only bring up one issue at a time
7. Be specific – focus on specific points or behaviors that will help the person change for the better
8. Do give positive feedback
9. Do challenge when necessary
10. Use "I" statements rather than you statements
11. Notice the response/feelings of the other person

Receiving Feedback

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1. Listen and hear what is being said – Do not assume you know what the other person is going to say, so wait to respond until you hear what has been said
2. Do not interrupt or talk over
3. Make sure you understand what has been said – clarify the feedback and if you don't understand, ask
4. If you feel emotional about the feedback, it may be better to reflect on it after the session before saying anything
5. If you receive similar feedback from more than one person, it usually means that people are noticing something important about you



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MODULE 3: Introduction of Key Concepts

Key Learning Points/Module Goals

- To provide individuals an understanding of risk assessment and risk factors
- Introduce cognitive distortions, treatment models, and relevant law

After a brief check-in, the facilitator should ask for volunteers to present homework and key concepts will be written on the whiteboard. All group members are expected to participate to some degree and reinforcement should be provided. Group members are also encouraged to ask others and/or reinforce the points mentioned by other members. The list of social activities should be discussed and encouraged throughout the treatment program, particularly during or right after the check-in to ensure this behavior continues.

The facilitator should start this discussion by asking group members about their understanding of sexual offender treatment including what they see as important elements, what they want to be included in the program, etc. It will be important to reiterate here that this specific curriculum is based on a model that has shown tremendous success, not only in reducing risk for re-offending but in improving lives beyond sexual aggression.

Types of Risk Assessments and Risk Factors

Clinicians should introduce the concept of risk assessment and present information related to risk factors (static, stable, and acute). **The reason that we use these tools is because no one is very good at predicting what another person is going to do just based on our impressions or knowledge of that person.** No risk assessment tool is perfect, but the ones we use are designed to be combined so that we get a better picture of someone's risk for re-offense. Identifying risks factors will also identify protective factors (strengths) that can help decrease a client's risk. This information will help clients and therapists in understanding the best way to address specific treatment needs and to create a focused treatment plan.

After the discussion/presentation of risk assessment and risk factors, facilitators could ask for volunteers and generate a discussion on what the clients see as their risk factors. As this is a preparatory program, facilitators should focus on areas that the clients feel they

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are doing well or have made some changes in. For example, a client who recently joined an organization or has established a healthy relationship.

Risk Assessments

The following static and dynamic risk assessments are used by the HOPE Program to identify a client's treatment needs. An agency called SARATSO (State Authorized Risk Assessment Tools for Sexual Offenders) has identified some tools that we have to use with every client. We use additional risk assessment tools to get a clearer picture of each client's needs and treatment goals.

STATIC-99R

The Static-99R is a tool designed to assist in predicting the risk of sexual and violent re-offending. It's referred to as 'static' because it scores people in areas where little, if any change is possible. This tool is scored using paperwork (rap sheets, probation reports, court documents, etc.) rather than from gathering information from clients through interviews. The instrument is used all over the world and has been tested over and over again to make sure that it is an accurate predictor of risk for re-offense.

In California, the Static-99R is scored by trained employees in prisons and jails. Everyone who has been convicted of a sexual offense is scored on the Static-99R before they leave custody, no matter how long ago the offense occurred or why the person was in prison this time. A client's Static-99R risk score was calculated while in custody and given to treatment providers to assist with risk assessment. The Static-99R has shown moderate accuracy (better than clinical judgment but not perfect) in ranking clients according to their risk for sexual re-offense. It's accuracy in assessing risk has been consistent across a wide variety of groups of people, in many different countries and in many different settings.

The 10 things that get scored on the Static-99R are:

1. Age at time of release from last sexual offense
2. Ever in a romantic live-in relationship for two or more years

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3. A violent offense that occurred at the same time as the sex offense (i.e., rape and robbery)
4. Violent offenses prior to the sexual offense
5. Number of sex offenses prior to the last offense
6. Number of sentencing dates prior to the last sexual offense
7. Any convictions for non-contact sexual offenses (indecent exposure, child porn, etc.)
8. Unrelated victim(s)
9. Stranger victim(s)
10. Male Victim(s)

Scores range from -3 to 12. There are five risk categories:

Level I – Very Low Risk (-3 to -2)
Level II – Below Average Risk (-1 to 0)
Level III – Average Risk (1 to 3)
Level IVa – Above Average Risk (4 to 5)
Level IVb – Well Above Average Risk (6+)

For CDCR clients, Static-99R risk scores is the main way that Parole determines the intensity (number of treatment sessions) for each client – in general, a score of 4 or above requires a client to be in a twice-a-week group and once-monthly individual therapy. Scores of 3 or lower are generally required to attend group once per week and individual therapy once per month. These are the guidelines but there are many exceptions that can change the amount of treatment someone receives.

STABLE-2007

The STABLE-2007 is another risk assessment tool that can help us predict the level of someone's risk to reoffend sexually. The difference between the STABLE-2007 and the Static-99R that we just described is that the Stable looks at changeable risk factors to estimate risk of sexual re-offense. Treatment providers use the STABLE-2007 as one way to identify treatment goals.

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There are 13 areas that get scored on the STABLE-2007:

Significant Social Influences - Who is the client close to? Are those relationships positive (healthy, non-criminal), negative (unhealthy, potentially pro-criminal), or neutral (neither positive nor negative)?

Capacity for Relationship Stability – Has the person lived with a romantic partner continuously for two or more years? Is the person currently in a relationship? If so, is the relationship healthy or unhealthy? If the person is not in a relationship, is he lonely?

Emotional Identification with Children –This is scored only for clients who had at least one victim who was 13 years of age or under. Does this client prefer the company of children to that of adults? Is s/he more comfortable talking to kids because they are less judgmental and more open? Does this client lack adult friends?

Hostility Towards Women – Does the client have female friends who he is not sexually involved with? What does the client think about women in positions of power? Does the client believe that women in general are not to be trusted and go out of their ways to get men in trouble? This question is looking for *generalized* hostility towards women, not just anger at a specific woman.

General Social Rejection and Loneliness – Is the client isolated and lonely? Does s/he feel rejected by his or her family or community? Does this client have difficulty making and keeping healthy friendships and relationships?

Lack of Concern for Others – Does the client express concern about the needs and feelings of others? Does he take those needs and feelings into account when making decisions and acting? Does the client have a group of people that he feels close to and cares about, and whose needs have an influence on his choices?

Impulsive Acts – Is this client prone to being impulsive in multiple areas of his life? Does he demonstrate difficulty with thinking something through before he acts? Does he seek out high-risk/high-thrill activities? Does he have a history of quitting jobs without having another one lined up? Does the client get into frequent fights? This item is looking for generalized impulsivity, not just impulsive behavior in one area of his life.

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Cognitive Problem-Solving – How good is the client at solving problems in his or her life? Is he or she able to think through and evaluate the problem so that it's clearly understood? Can he or she come up with ways to solve the problem, evaluate the different ways, and choose the one with the best possibility of a good outcome?

Negative Emotionality/Hostility – Does the client feel like a victim? Does he or she feel like the world has treated them unfairly more than most people? Does this client brood on that unfairness and feel angry and hostile about all the ways that things have been unfair?

Sex Drive/Preoccupation – Does the client have an unusually high sex drive for a person his or her age? Is this client preoccupied with sex (thinks about sex all the time, might have a history of making people uncomfortable because of sexual joking, has a large porn collection, spends a lot of time thinking about and planning sexual activity)? Does this client spend time masturbating or looking at porn to the point that these activities interfere with relationships or jobs, or cause physical harm?

Sex as Coping – Does the client use sexual activity (masturbation, porn, sex with others) as a way of managing negative emotions? Does the client regularly masturbate to make him or herself feel better, or to avoid feeling sad or lonely?

Deviant Sexual Arousal – Is the client aroused by the idea of sexual behavior with a person who can't consent (a child, a dependent adult, someone who is sleeping or drunk) or who didn't consent (sexual assault or rape victim)? This is scored based on history (who the victims in the client's offense were, and how many sexual acts in which client engaged), on self-report (someone who admits to having interest in non-consensual sexual activity) and on testing (PPG or Abel – we'll talk about these later)

Cooperation with Supervision – Does the client cooperate with his or her Parole agent or probation officer? Has the client been violated for not complying with conditions of supervised release? Is the client cooperative with treatment providers?

Areas on the STABLE-2007 where the client scores '0' are considered 'protective,' or strengths – these are the areas in a client's life where he or she is doing well. These strengths can help the client work on areas where there is more challenge.

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For each client, those items on this tool that are scored '2' are immediate treatment goals, as they are the areas of greatest risk for the client, and if changed, can decrease risk for re-offense.

Level of Service /Case Management Inventory (LS/CMI)

The Level of Service Case Management Inventory (LS/CMI) is a risk and need assessment tool that helps us assess each client's risk for getting into trouble with the law for any reason – it's not specific to sexual offenses and can be used with anyone who has been arrested for a crime.

Researchers have spent a lot of time identifying things (referred to as "factors") that increase people's risks for getting in trouble with the law. What was found after a lot of research is that we can identify the most common problems in the lives of people who commit one crime, and then, go on to reoffend. Those are referred to as "The Eight" risk/need factors: The most important of the four (meaning they have the most impact on a client's risk for committing another crime) are Criminal History, Antisocial Companions, Pro-criminal Attitude, and Antisocial Pattern. The other four factors that are important are Education/Employment, Family/Marital issues, Leisure/Recreation, and Alcohol/Drug Problem. Just like with the STABLE-2007 that we just finished talking about, areas where a client has a low score are considered protective factors.

Like the STABLE-2007, HOPE uses scores from the LS/CMI to help set treatment goals as most of the things that are assessed by the LS/CMI are changeable – in other words, if a person makes positive changes in these goal areas, their risk for getting in trouble with the law decreases.

The Eight Risk/Need Factors on the LS/CMI

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Criminal History – This is a 'static' factor; we can't go back and undo someone's criminal history. It makes sense that having a long history of problems with the law makes it more likely that a person is at greater risk for committing another crime.

Antisocial Companions – The more time someone spends with other people who are breaking the law or engaging in risky behavior, the higher the chance of that person getting in trouble again. It's much harder to stay "clean and sober" if you are spending most of your time with others who are still drinking or using. In the same way, it's much harder to avoid crime or risky situations if you are hanging out with people who are still committing crimes.

Pro-criminal Attitude - Pro-criminal attitude is used to describe someone who thinks it's ok to break the law and to not follow the rules of society. Are the person's values and the ways s/he thinks about her/himself and others prosocial (supportive of social rules and laws that enforce positive social behaviors) or antisocial (indifferent or hostile to following the rules)?

Antisocial Pattern – This factor looks at when a person first broke the law, overall criminal history, whether a person has ever been able to live a non-criminal life, history of violence, history of stability in the community, etc.

Education/Employment – A person's employment history is important. The LS/CMI wants to know if the person is employed full-time right now – being unemployed or underemployed leads to lots of other problems like financial stress, too much time on a person's hands, etc. Going to school full time counts as full-time employment. Level of education has an impact on the kinds of job a person is able to get. How well a person does at work or school (relationships with bosses and co-workers or fellow students) is some indication of how well they will do at work or in school.

Family/Marital – Social relationships are very important to most people. Having satisfying relationships with romantic partners, parents, and close relatives makes it less likely that people will get in trouble with the law because they're part of a community and are more likely to get feedback about how positive and negative behaviors impact people they care

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about. People who have family members who have a criminal history are more likely to continue to get in trouble with the law.

Leisure/Recreation – "Idle hands do the devil's work" is a quote you might have heard. People who have a history of criminal behavior are more likely to get in trouble again if they don't make productive use of their free time. Having hobbies and recreational activities that are satisfying and interacting with others in an organized way during time off work or school decreases the risk for using that free time in negative ways. Organized activities can mean team sports, volunteer work, belonging to clubs – whatever helps a person make positive, social use of free time.

Alcohol/Drug Problems – It's fairly obvious that having problems with substance abuse increases people's risks for negative, possibly criminal behaviors. A history of substance abuse increases risk a little bit. Current abuse of substances, and negative impacts; such as, relationship stress, illegal behaviors (e.g., DUI, etc.), or problems at work or school increase risk a lot more.

Cognitive Distortions/Thinking Errors – A brief introduction



There is one risk factor that isn't assessed by the tools we described above – cognitive distortions – what we general refer to as "thinking errors." We'll talk about cognitive distortions and thinking errors in depth later on in treatment, but for right now, here is a simple definition of cognitive distortions/thinking errors:

"Thoughts or beliefs that distort a person's perception of reality, often in a negative way."

We all use cognitive distortions all the time to convince ourselves that it is okay to do things that we know we shouldn't be doing (thinking errors like minimizing something or rationalizing bad behavior or poor choices). Cognitive distortions cause us to not see things clearly and to make poor decisions based on our inaccurate perceptions. An

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example of a cognitive distortion is “black and white (polarized) thinking.”’ This way of thinking can lead us to see things as “entirely good” or “entirely bad,” neither of which is accurate as many situations are actually some of both, or “gray.”

Treatment Models and Relevant Laws

Present and discuss the predominant sexual offender treatment models. These include:

- Risk, Need, Responsivity (RNR) model
- Relapse Prevention Model and CBT
- Self-Regulation Model
- Good Lives Model
- Containment Model
- Relevant laws (e.g., Jessica’s Law)

Risk/Need/Responsivity Principles

- **RISK**: The *intensity* (how often, for how long) of each person’s program should be based on his risk level – this is figured out by using risk assessment tools, which we’ll explain in a future module.
- **NEED**: Each person’s specific treatment needs, also known as *dynamic risk factors*, should be identified and treated. The risk tools we use help us figure out what needs to be addressed in treatment in order to reduce risk.
- **RESPONSIVITY**: Treatment should be provided that best fits the person’s learning style, ability, and personal circumstances - this could include language, culture, personality style, motivation, anxiety, and cognitive abilities.

Relapse Prevention (see model presented in appendix)

Relapse Prevention Therapy is a ***cognitive behavioral approach*** that was originally designed for treatment of addictive behaviors such as substance abuse.

Cognitive Behavioral Therapy (CBT) explores the relationships between people’s feelings, thoughts, and behaviors and works to help people become aware of inaccurate

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and/or negative thoughts in order to help review situations more accurately and respond more effectively.

- At HOPE, CBT is used as a way to teach clients how to manage their emotions and alter their thoughts in order to recognize and cope with triggering events, situations, and feelings that are likely to lead to a negative behavior (or relapse, which means another sexual offense).

- Even though we often tell ourselves that things “just happened,” sex offenses, like drug or alcohol problems, never “just happen." There are always a number of steps that people go through before they relapse or commit an offense, also known as the Offense Chain. Understanding the relapse process is useful – by knowing the steps that lead to unhealthy or illegal behaviors, we can come up with strategies at each step in the process that could prevent us from moving to the next step (meaning closer to a relapse or an offense). CBT helps people recognize high risk thoughts and feelings before they turn into high risk situations that can result in another offense. It can help people see how a seemingly unimportant, non-harmful thought or decision can start them on the path to another offense.

Self-Regulation Model

The Self-Regulation Model is different from the traditional Relapse Prevention model in two important ways. Instead of focusing completely on avoiding high-risk thoughts, feelings, or behaviors, the Self-Regulation Model focuses on developing goals for living a positive, crime-free life while also paying attention to risk factors. Also, the Self-Regulation Model differs from Relapse Prevention by showing that there can be multiple pathways to offending and different strategies used to meet negative goals. This differs from Relapse Prevention, which only looks at one pathway.

The Self-Regulation Model uses avoidance, like Relapse Prevention, but it is also *approach*-oriented, meaning that it helps people move TOWARD (approach) something they want.

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By using both the Relapse Prevention Model and the Self-Regulation model, the HOPE program provides individualized treatment that focuses on strengths and risk factors instead of a single treatment pathway for all clients.

Good Lives Model

While both the Relapse Prevention and the Self-Regulation approaches to treatment for people who have committed sexual offenses require that people look BACK at their offenses and learn to understand the risk factors (thoughts, feelings, fantasies, areas of personal challenge, and situations) that resulted in a sexual offense, the Good Lives Model focuses more on looking FORWARD. The model is strength-based (meaning that it helps people identify their strengths rather than just looking at challenges) and is based on the idea that people who commit sexual offenses may be violators of human rights but also have human rights themselves. The model suggests that humans require positive things/experiences in a variety of areas of life in order to feel whole. Those areas include Life/Healthy Living, Knowledge, Excellence in Work and Play, Excellence in Agency (self-determination and self-agency), Peace of Mind, Relatedness (family/friends), Community, Spirituality, Happiness, and Creativity. The model helps clients understand that their needs in the areas above are normal but that the ways in which they tried to meet some of those needs were harmful to others and themselves.

The HOPE Program uses the Good Lives model to help clients and therapists understand and evaluate what risk factors might be getting in the way of clients' abilities to have positive things in the areas of Human Good. That understanding is used to help clients set goals in those areas and develop prosocial (non-criminal) strategies for achieving those goals in order to increase the good things in their lives. We expect that by teaching clients how to acquire positive things (Good Lives Goals) in non-criminal, non-harmful ways, we will encourage clients to attain their Good Lives Goals without harming or sexually offending against others (Yates, Kingston, and Ward, 2009).

The combination of Relapse Prevention, Self-Regulation, and Good Lives Model in treatment helps clients recognize, understand, and if necessary, change thoughts, feelings, and behaviors that put them at risk for committing another sexual offense, while at the same time, giving them tools to live a happier, healthier life. In other words, the program addresses client's risk while helping them improve their lives.

The Containment Model and Laws Governing People Convicted of Committing a Sexual Offense

The Containment Model was created to increase community safety by helping Parole Agents and Probation Officers work together with treatment providers to provide the most effective assessment, treatment, and supervision strategies for clients based on their level of risk for committing another sexual offense. The model is based in research that helps us figure out what works best in reducing recidivism. Basing what we do on research is really important – we need to be able to prove that what we do works. We know that if we can support each of you in getting through the treatment program, we can help you reduce your risk for re-offense by up to 50%!

It is a collaborative approach to working with people convicted of sexual offenses after they've been paroled into the community. On July 1, 2012, the Containment Model became mandatory in California. This means that all Parole units and all County Probation officers, along with all certified treatment programs, are required to use this model when working with clients who have been convicted of a sexual offense. The Containment Model requires programs to have standards; such as, training and experience in sex offender specific treatment. Such standards ensure that treatment professionals are qualified and use an effective, evidence-based approach to sex offender specific treatment. You'll hear us use the term 'evidence-based' while you're in treatment with us. What this means is that the material we are sharing with you has been shown to help people reduce their risk for re-offense. We'll explain more about this when we talk about how we assess each person's risk for re-offense.

The Containment Model requires that all clients be scored on several different risk assessment instruments when entering sex offender treatment. We'll explain each type of risk assessment in the next module. The new law allows the length and frequency of attendance to be tailored to each client's risk level.

The Containment Model uses a victim-centered approach, meaning that the primary responsibility of all parts of the containment team is community safety and reduction in clients' risk for re-offense. The three main people or groups who must have regular communication about the person on parole or probation are:

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1. The supervising officer (Probation Officer or Parole Agent)
2. The HOPE Program therapist(s) involved in providing assessments and treatment
3. The polygraph examiner

These three people or groups are the core of the Containment Team, although other team members should participate at times to share general information (for example, the law enforcement agencies who are responsible for 290 registration, or victim advocates).

During your first meeting at HOPE, you were asked to sign releases of information (permission for us to communicate with others on the Containment Team), allowing the people listed above to openly communicate about any concerns and about your progress in treatment.

The HOPE Program is responsible for making sure that you complete a comprehensive evaluation that includes several risk assessment tools. Your risk scores, cooperation with treatment and program policies, polygraph results, progress in the program, and/or any concerns regarding your risk will be communicated to your supervising officer.

As a reminder, the treatment contract you signed on the day you started treatment at HOPE describes your responsibility in avoiding risky, aggressive, or re-offending behavior and high-risk situations. The treatment contract requires that you report any such behaviors to the HOPE Program and supervising officer as soon as possible.

Other important laws

There are a number of laws that have an impact on people who have been convicted of sexual offenses. Some of those laws apply to all registered sexual offenders (290 cases) who live in the community, and some laws apply only to registered offenders who are currently on supervised release. These include Jessica's Law, Megan's Law, Chelsea's Law, and registration. The facilitator should present a broad overview of such laws and generate a discussion about individual's experiences with the overall tenets of the laws. Going into considerable detail of each law is not recommended.

Introduce the core elements of the Prep Program as well as the core HOPE program

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Discuss the core elements of this program as well as the core HOPE sex offender treatment program (see manual)

Introduce the key assignments of this program (e.g., wheel of life; autobiography and offense disclosure) as well as other key concepts/sessions that relate to motivational enhancement, goal setting, and engaging in therapy. Emphasis can be placed here on the fact that this program is designed to flow into the main program and that research shows that individuals who put forth a lot of effort in this initial phase of treatment do much better in the core treatment program and after they are done with treatment. Bring up the fact that there will be repetition but that this is by design and contributes to successful outcomes.

Closing the session

Review the major points of the session (program overview). Heavily reinforce participation of group members, even if minor participation. Provide a brief discussion about what they can expect in the next session (GLM) as well as subsequent assignments (e.g., autobiography).

Goal Setting Homework



Each member should write up to three goals they want to achieve in the program. This could be something they want to change in themselves, something they want in a better life, etc. Emphasize that these will be discussed next session.

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MODULE 4: MOTIVATION AND ENGAGMENT

Key Learning Points/Module Goals

- Present psychoeducational information regarding motivation and engagement
- Introduce concept of resistance and the importance on change
- Discuss and implement goal setting

An essential goal of treatment, particularly during the early phases, is to build motivation to change and to engage group participants in the treatment process. Research has consistently shown that client motivation is an important determinant of treatment outcome.

Although motivation is presented here as a specific module, it is expected that motivational enhancement is integrated throughout the treatment program. As such, group leaders must remain mindful about opportunities in every session to increase and/or maintain motivation to change. Group leaders should refer to the important elements of motivational interviewing as well as the therapist characteristics that are essential to motivational enhancement and establishing a strong therapeutic alliance. Group leaders should also be familiar with Prochaska and DiClemente's (1984) Transtheoretical Model of Change. There are other excellent resources available on motivational interviewing that would be helpful in delivering this session and in working with client motivation in general (e.g., Miller & Rollnick, 2012). Facilitators then introduce and describe the Good Lives Model in more detail, building on the earlier description and highlighting how this can help participants to achieve their goals.

Sessions 6-11

** Session 5 commences with a check-in and reviewing the goal setting homework

Prochaska and DiClemente's Stages of Change Model

The facilitator should introduce the Prochaska and DiClemente's Transtheoretical Model of Change. The model should be displayed on a whiteboard or handout and an explanation of each stage is provided.

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The Stages of Change

1. Precontemplation – not thinking about changing
2. Contemplation – considering changing
3. Preparation – getting reading for change
4. Action – making efforts to change
5. Maintenance – sustaining change

Pre-Contemplation: During this stage individuals have not yet identified that there is a problem with their behavior. Typically, other people are aware that the person is having problems. However, people with behavior problems are usually so rooted in their Treatment Readiness Block Page 21 of 43 behavior that they easily turn a deaf ear to the concerns of others. A person in the precontemplation stage is "uninformed" because he has yet to be presented with a convincing reason to change. An example might be an individual who believes that they can stop doing heroin anytime and will not experience any negative consequences. Another, more immediate example is someone who gets into his car drunk thinking "I can drive fine when I am drunk." In both of these examples, the individual is not even considering that there is a behavior problem.

Contemplation: Once the individual enters into a dialog (even a healthy debate) regarding the merits of his behavior and considers the possibility of change, he has entered the contemplation stage of change. At this stage of change the individual often vacillates between the idea of changing and defending his old behavior. Despite the desire, some clients feel that they are unable to change and that their old behavior is their only choice. However, the ability to contemplate change allows the change process to be owned or personalized as opposed to being imposed. If an individual is not convinced that changing is better than staying the same any change that is made is not likely to be true or long standing. Deciding whether to change or not is usually determined through an evaluation of the pros and cons of changing. This is called DECISIONAL BALANCE. In other words, which way do the scales tip, toward changing or not changing? There are two ways to determine decisional balance. The first is doing a "cost –benefit analysis" regarding the behavior.

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Preparation ("Getting Ready"): In the preparation stage, the individual is no longer ambivalent about making changes and makes serious efforts to develop strategies to correct behavior. At this point, a client is seriously thinking about change and also beginning to take some action. In effect, he is ready for change as he has resolved the indecisiveness of the contemplation stage. The individual can visualize goals and success. Individuals may express a desire for direction and insight from others. During the preparation stage individuals may experiment with change processes such as "trying out" a group or making mild disclosures. In some cases, individuals will report past attempts at changing and explore the lessons learned from these past attempts (DiClemente et al., 2001). This is a turning point at which the person decides to change or quit the behavior.

In order to approach this stage in a conscious and useful way, the individual must be able to identify what it is about their lifestyle that they want to change. According to this model, it is important to understand that the individual doesn't need to change every aspect of his life. It is important however, to identify strengths and problem areas. The individual must view the goal as conducive to his capabilities, needs, and values. He also benefits best when he sees the goal as reasonable. If the goal is unreasonable to the client, or beyond his personal expectations he is likely to become discouraged and fail.

Action: Graduation to the action stage is measured by observable lifestyle changes. This includes an obvious investment in the change program as well as concrete changes in one's everyday life. The action stage challenges individuals to abstain from past behavior and adopt new ways of being. In the case of clients who have engaged in sexually abusive behaviors, this would mean cessation of high-risk pre- Treatment Readiness Block Page 22 of 43 offense offence behaviors. Such changes would likely relate directly to the variables that contributed to problematic behaviors.

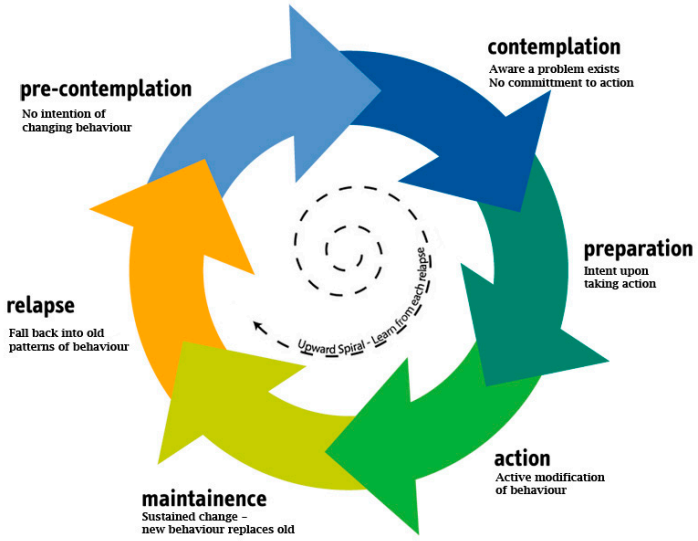
Maintenance: The maintenance stage is attained when the individual has been successful at replacing undesirable behavior with healthy positive behavior. However, this stage also acknowledges that there is a history of problematic behavior and steps need to be made to foster ongoing beneficial behaviors. Like a good running car, maintenance is required to assure both its current wellbeing and longevity. Relapse into problematic behavior

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needs to be prevented by remaining cognizant of the strategies that prevent harmful and self-defeating behavior. Achieving success without challenges is rare.

Each stage represents the state of the individual and his decision-making process cognitive experiences, behavior, and emotions as opposed to external or social influences of change. In addition, the stages are not necessarily linear, that is individuals may demonstrate signs of advancement onto the next stage only to return to a previous stage.

For this reason, Stages of Change usually takes the form of a "spiral." That is, some individuals will move linearly through the stages of change from contemplation to preparation to action and then to maintenance, but most individuals will spend time in one stage and then regress to an earlier stage before finally advancing. Movement between stages results from a struggle between the pros and cons of changing, or more commonly referred to as Decisional Balance (Janis and Mann, 1977). Individuals should be supported through this process so that demoralization and increased resistance can be averted (Prochaska & DiClemente, 1992). Ideal progression through the stages will accompany increased awareness of triggers and potential lapses to earlier stages.



Transtheoretical Model of Change
Prochaska & DiClemente

Exercises related to Stages of Change

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- Have members share personal examples of change. If there are no volunteers, using “ice breaker” examples of other cases. Ask volunteers to share their experiences of change and how they were able to progress. Remember, as this is a prep program, greater attention should be devoted to areas of strength and success although discussion of times when things did not go as planned can be beneficial.
- The Decisional Balance Exercise handout is provided to participants (See appendix). Each group participant should volunteer one behavior that they want to change and complete the exercise on a flipchart. A general discussion should ensue regarding the following
    - The importance of change to participants
    - Participants’ confidence in their ability to change
    - Participants’ reasons for change at this time in their lives, including why they want to change now
    - How changing now can be good for the future

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Motivation and Resistance

Participation in therapy is critical to understanding the underlying motivations for your sexual offending behavior and to learn more adaptive ways to manage these underlying factors. However, in order to do this a person must make a commitment to honesty, disclosure, and be willing to face the possible challenges confronted during therapy. Depending on your level of commitment to the program, you may experience treatment as a very restrictive, controlling and manipulative dilemma, especially in cases of court ordered therapy.

Common stances may include:



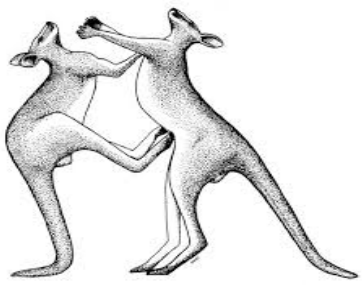
- Motivated and compliant: "I will do anything that will help. I need this."
- Resistant and vocal: "You are trying to control my whole life. This is worse than prison. This is brainwashing, mind control, Clockwork Orangesque, etc."
- Pseudo-compliant: "Sure, sounds OK, I'll sign it (but I'll be damned if I will do it.)"
- Vacant, uncomprehending: "Where am I supposed to sign? What is the date today?"
- Litigious: "You don't have the right to do this to me. I want to talk to my attorney first. Who makes the Abel Screen and has it been approved? You can't force me to incriminate myself."

Thus, in order to move forward in therapy it is important to understand for yourself whether there are forces causing you to resist becoming involved in therapy or motivating you to be involved in therapy.

Naming the forces – Motivation and Resistance

Acknowledge some important realities about motivation and resistance:

- There are motives or forces that are pushing each of you to really engage here, to do your best, to tell the truth, to identify the problems, to begin to make changes in the way you think and feel and act and live your life.



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- There are also forces pushing the other way: do the least you can to get by, get out as soon as possible, don't let anybody really see who you are inside, don't change your life, don't tell the truth about yourself and what you did.

For some of you this struggle will go on for a while and it isn't clear which side will win. Your clinicians will probably get involved in your struggle to some extent, though only you can decide its outcome.

For now, we just want to name the forces at work.

Experiential Exercise



Facilitators Draw a scale (a balance) on the board (or perhaps a rope to represent a tug-of-war) and begin to put ideas on each side of the scale as the group develops them. Make a distinction between the forces that are "inside" of them and those that are "outside" of them. (Of course most of the external forces are on the side of engaging in the work.) Start with the negative: what are the reasons or the forces pushing you to not get invest in treatment; “get to work”? Elicit ideas from the group on the forces they experience that make it hard to commit to treatment this work.

Some that should emerge are:

- Mistrust of others finding out
- Shame about what I have done and about myself
- Fear of what others will think
- Anger about being forced to come here
- I already did my time
- May make me anxious, depressed, upset
- I am used to hiding a lot of things about myself
- Only I know what is best for me
- What I am supposed to talk about is too painful
- I hate to be told what I to do
- I learned my lesson so I don't need this program (I already did my time)
- I didn't do what they said I did

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Then move to the positive, pro-commitment side:

- May get into more trouble if I don't
- I want to understand more about how I could have done this
- To tell the truth would feel really good
- Things can't get any worse
- I am afraid of being confronted
- It is time to face the problems in my life
- I owe it to my victim or my family
- It is the only way I can have a better future
- I never want to do anything like this again
- It is the right thing to do

Participation in therapy does not mean that you just come to the groups and listen. Simply understanding the material on an intellectual level is not enough. You must demonstrate that you have internalized what you have learned and integrate it into your life. This treatment program cannot "cure" you, so therapy will not work for you unless you are willing to make a commitment to put effort into it.

Remember that you can, if you want to, make the most of what is available here for yourself. You can come out of treatment – as many men have before you – a changed man, a man who feels a lot different inside than you do now, a man really in control of himself and his life, a man who feels some pride in himself and what he has accomplished. It is up to you.

Motivation and Resistance Homework



All of this may sound really demanding, and in a way, it is. So, let's take some time to look at what motivates us and where the obstacles are. Each member is to write down a list of reasons why someone **should not** participate fully in the group and **should** participate fully in group. This includes fears of being judged or being confronted with an offense that has not yet been identified. Then go back and underline the reason or circle the number that applies directly to you. **Each member should come up with at least 15 reasons** on each side of the scale. Since we've started this exercise in group is should be easy.

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Why someone <u>should not</u> participate fully in the group:

1. _____
2. _____
3. _____
4. _____
5. _____
6. _____
7. _____
8. _____
9. _____
10. _____
11. _____
12. _____
13. _____
14. _____
15. _____

Why someone <u>should</u> participate fully in the group:

1. _____
2. _____
3. _____
4. _____
5. _____
6. _____
7. _____
8. _____
9. _____

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10. _____
11. _____
12. _____
13. _____
14. _____
15. _____

Goal Setting/GLM

Facilitators should discuss the importance of goals with the group and link it back to the earlier goal setting exercise.

Review the GLM good live goals and the goals identified on the Wheel of Life and discuss the difference between short-term goals and long-term goals. Generate a discussion with group members regarding their experiences with setting goals. Also, the facilitator should be mindful of the difference between avoidance goals and approach goals. Clients often frame their goals as avoidance-based (i.e., things they should not do) such as avoid children and certain places. While this is appropriate and deserves some attention, considerable discussion needs to be placed on approach goals (i.e., things they will do). Examples of approach goals are a) establish a healthy relationship and b) establish a group of prosocial friends.

Wheel of Life Exercise



The therapist should hand out copies of the Wheel of Life (see appendix). The analogy of the wheel is used to reflect the notion that a well-functioning wheel should be large and round. The smaller the wheel or more irregular it is, the harder it is to function properly. It is also good to note that most people don't have a perfect wheel but that it is important to recognize the areas in need of improvement. The therapist should also indicate to the clients that the goal of treatment is to equip them with the skills and self-confidence in each area of the Good Life Model so that they will have a better and more satisfying life. Success in at least some of these areas will help ensure risk to re-offend is reduced.

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Have each individual (between session) assign a value from 0-10 indicating the degree to which he feels satisfied with his achievement in each of the nine areas of functioning. Ratings can be done both for how they feel currently and for how they felt during the time of the offense. Any differences should be discussed, particularly as individuals advance in the program.

How to Achieve Goals



Building on the discussion of the homework exercise (Wheel of Life), facilitators discuss with participants how they can achieve the goals they identified. It is important to facilitate the development of goals derived from the Good Lives Wheel, that a discussion ensues about the importance of setting goals and the process of doing this. Therapists should ask for examples of previous goal setting attempts and how and why these were successful or unsuccessful. Referring to each client's Good Life Wheel, the therapist and group can identify areas where goals can be described and strategies to obtain these goals can be identified. The SMART (specific, measurable, achievable, relevant, and timely) framework for goal setting can be used for this purpose.

Participants should generate approach goal ideas to achieve their goals, and the facilitator reinforces these goals and methods. Facilitators need to ensure not to overly reinforce too many avoidance goals on the part of participants, as these are more difficult to maintain over the long-term but should ensure to reinforce such avoidance goals as preventing gaining access to victims, in addition to positive approach goals.

Facilitators note that all people have goals, and that they must actively work toward achieving these goals. In this exercise, participants generate concrete, specific ways to start working toward achieving their goals. These are recorded on the flipchart and are placed on the wall of the program room as a tool to remind participants throughout the program to use these strategies. Examples include participating actively in treatment, identifying smaller "sub-goals" that are more immediately achievable, practicing achieving these smaller goals, and keeping track of successes and difficulties so these can be discussed, and strategies and skills developed over time. These should also be expanded throughout the program as participants generate new ways to achieve goals and develop new skills.

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As an ongoing homework exercise, participants begin to self-monitor using these skills and strategies each day, with facilitators reviewing them in each subsequent check-in session.

Closing the Sessions

Review the major points of the session (the importance of goals, that people need to be motivated and actively work toward goals, that goals form an individual's good life plan, and that specific things can go wrong in attempting to achieve goals).

Make sure that the group members presenting their autobiography next are aware of this. Provide a brief discussion about what they can expect in next session. Close the session by "taking the temperature" of the group – are participants feeling positive, looking forward to treatment and the future and developing a good life plan? Are they feeling motivated?

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MODULE 5: Core Assignments #1: Autobiography/Life Analysis

Key Learning Points/Module Goals

- Presentation of autobiography/life analysis
- Begin to identify the relevant risk factors, treatment needs, and strengths
- Help the individual and group members gain insight into past choices and identify relevant distal factors

Autobiography and Life Analysis Assignment



Each client is asked to describe their life, including who he is now and how he came to be the person he is today. These assignments are completed between sessions. They can talk/write about anything they want but try to get them to include some of the positive things about their life as well as some of the negative things. Focus, in particular, on thoughts and feelings that they experienced at the times they are writing about. This exercise allows clients to identify with each other and become more aware of treatment targets that may be important for them to address in treatment. In addition, it allows clients to share personal details of their life history in a safe, supportive environment.

There really is no right way or wrong way to give an autobiography – it is their life. However, some discussion on childhood, relationships, major life events, and work history can help for identifying areas for development and planning for the future. If they do not mention all of these areas of life, ask them about them. Often what people leave out is as, or more, important than what they include. This exercise should be presented by the client in his own words and he should read this from his prepared written work.

Once the client has read his autobiography to the group, he should be provided with positive feedback from everyone, including the therapist. The therapist should focus on motivating and supporting each client during these discussions, which will facilitate trust and treatment engagement. These issues should be the primary focus, particularly in this early stage of treatment.

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While listening to the presentation of this assignment, the therapist should be mindful about the assets and deficiencies in stable dynamic risk factors as well as other factors that have contributed to vulnerability or resilience, such as early attachment to parents.

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MODULE 6: Core Assignments #2: Offense Disclosure

Key Learning Points/Module Goals

- Presentation of offense disclosure
- Begin to identify the immediate proximal factors associated with the instant sexual offense
- Identify relevant risk factors, treatment needs, and strengths

Offense Disclosure Module/Assignment



In this assignment, participants provide a disclosure of the sexual offence(s) that resulted in their incarceration and/or referral to this program. The purpose is to identify the affective, behavioral, cognitive, and contextual factors that culminate in the occurrence of a sexual offence and to assist clients in taking responsibility for their actions in offending. These sessions also allow clients to receive feedback from others in terms of their motivations and dynamics involved in their offence(s).

The purpose is to have each client identify the problems or other issues that occurred in the period just prior to his offense and that appeared to precipitate the offense. Taken together, the two assignments in this program (autobiography and offense disclosure) will begin to elucidate the distal and immediate factors that precipitate a sexual offense for that individual.

Facilitators should display the offense progression on the wall to discuss the relationship between distal and more immediate factors (see appendix).

Although some degree of supportive challenging is appropriate, it is fundamental that at this stage in the program, individuals are reinforced for their participation and their willingness to present any issue about the offense. It is also important to note that, as facilitators, we are most interested in the circumstances surrounding the offense (i.e., background/immediate factors) rather than the specific details of the offense itself.

** For individuals who deny their sexual offense, ask them to discuss what they were charged with. A discussion can then ensue about the circumstances surrounding when they were accused of the offense.

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APPENDICES

Therapeutic Processes

Recent research examining therapeutic processes with sexual offenders has shown that several features enhance the effectiveness of treatment.

I. Therapist features: empathy, warmth, rewardingness, directiveness, flexibility, and non-confrontational challenging

- *Empathy:* An empathic therapist is viewed positively by clients who accept his/her advice. Since we want our clients to develop empathy we must model it.
- *Warmth:* A warm and supportive therapist is viewed positively by clients who are then more likely to listen to and follow the therapist's advice.
- *Rewardingness:* Any signs of movement by the client in the desired direction should be encouraged as should reports of satisfactory between-sessions work.
- *Directiveness*: Clients should be encouraged to find their own solutions and to recognize their own problems but there are times when some guidance is required.
- *Flexibility:* This allows the therapist to meet responsivity needs. Each client has a different learning style as well as a different personal style, and the therapist needs to adjust his/her style to meet these differences. Similarly, each client may present at each session in a different mood or in a more or less cooperative mood, and again the therapist needs to adapt to this.
- *Nonconfrontation:* An aggressively confrontational style is not only perceived as denigrating by clients it also tends to make them respond in kind. A supportive but firm challenging style generates change.

II. **Group climate**: The two major features of the group climate that need to be achieved for maximum effectiveness are cohesiveness and expressiveness.

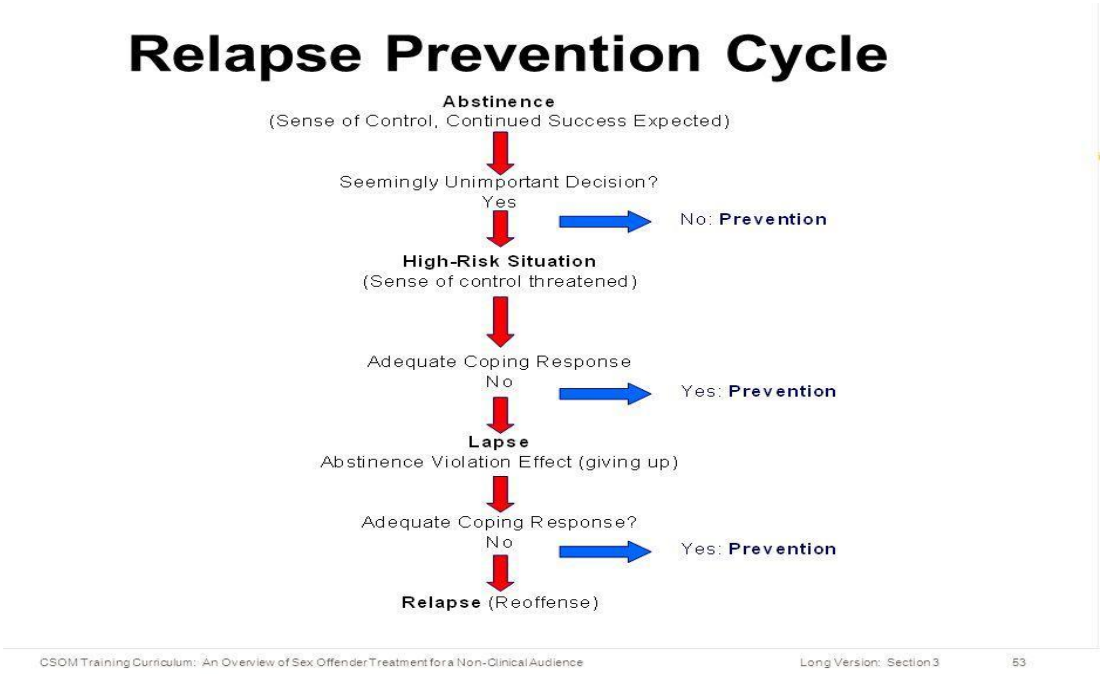
- *Cohesiveness:* When clients are supportive of one another while at the same time appropriately challenging, the goals of treatment will be met.
- *Expressiveness:* Having each client fully participate in all discussions and being free to express their emotions significantly influences outcome.

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III. **Client processes**: Clients must perceive the therapist as displaying the above mentioned therapist features. In addition, the client's intellectual grasp of the issues under discussion must be accompanied by appropriate expressions of emotions for changes to be made and retained.

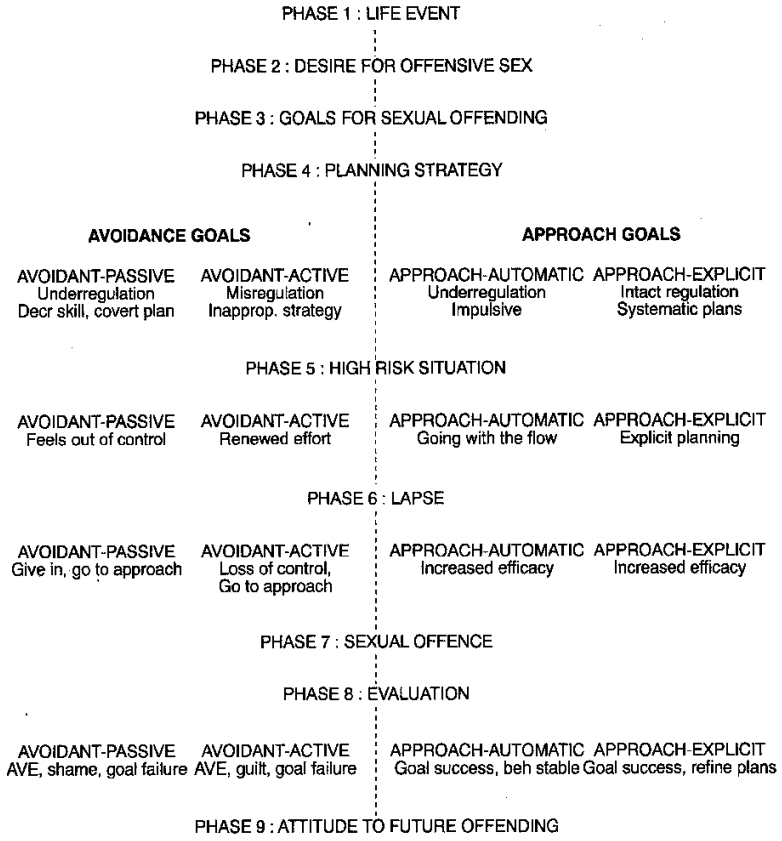
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Relapse Prevention Cycle



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Self-Regulation Model



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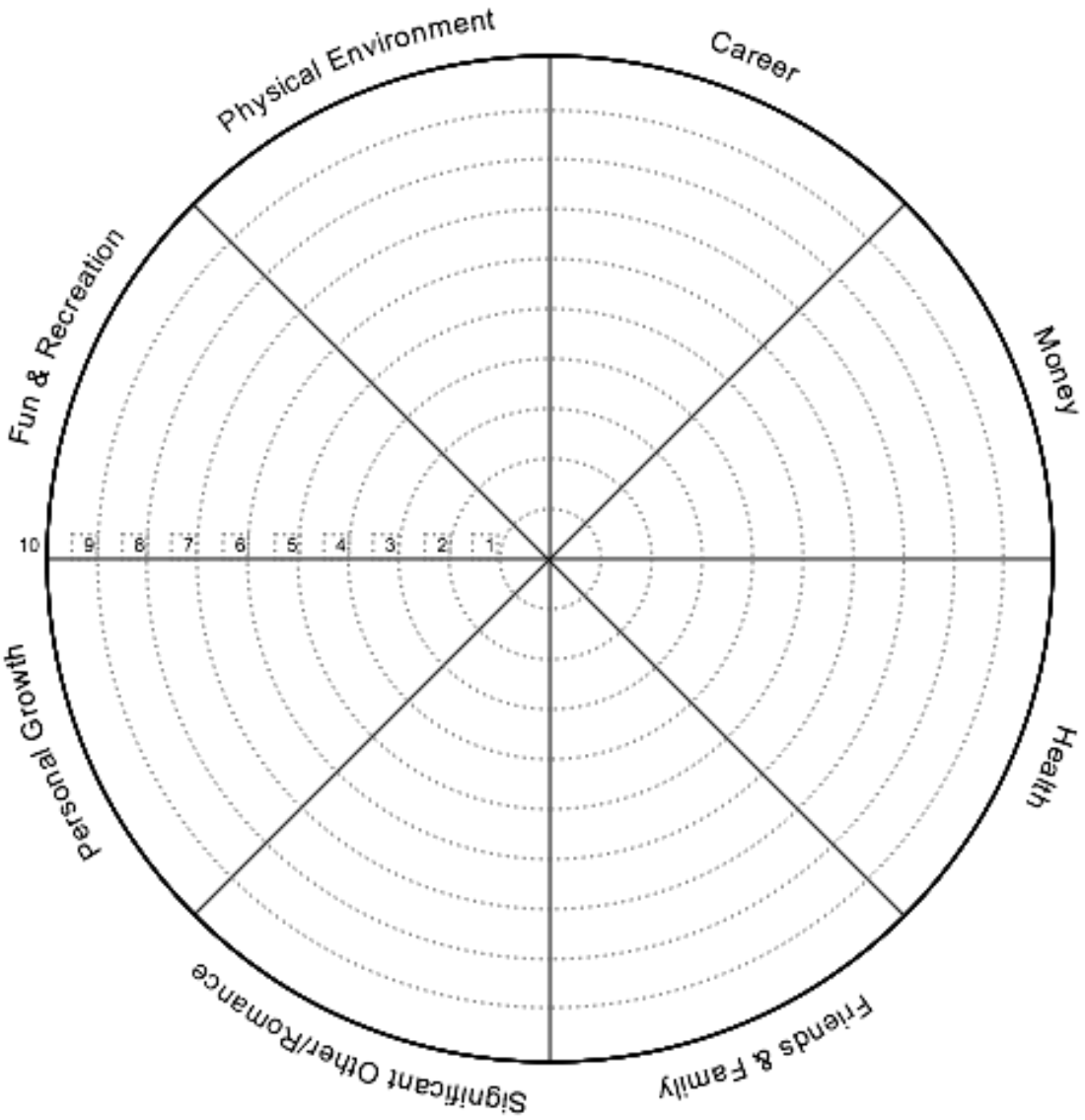
Decisional Balance Sheet

Name: _____

	Advantages	Disadvantages
Not Change		
Change		

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Wheel of Life Exercise



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Autobiography and Life Analysis Handout

PARTICIPANT HANDOUT **NAME: _____**

AUTOBIOGRAPHY and LIFE ANALYSIS

Everyone is unique and has a story to tell. This assignment provides you the opportunity to tell your personal life story. An autobiography is a description of your life from early childhood to the present time. The purpose of this assignment is to help us to get to know you better, including your strengths and prior challenges, and to help you understand how your past has contributed to you committing a sexual offense.

Using the space below and up to 5 additional pages, please write about your life. There are no right or wrong answers. You may want to include information about: Childhood and family; relationships with friends and family; romantic and intimate relationships, sexual history, substance use history; legal history, hobbies, interests, and leisure activities, and anything else you think would help us understand you better.

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Offense Disclosure Handout

PARTICIPANT HANDOUT **NAME:_____**

OFFENSE DISCLOSURE

Please tell us why you are in treatment/in this group? What are the charges or circumstances that brought you to this program. Details of what happened during the offense are not important but a reasonable account of the offense, what the charges were, who the victim was, time period over which the offense took place, how the offenses came to light and what was going on in your life at the time are very important!

Using the space below and additional pages as needed.

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